Novitas Health Seed Memo

Problem

In the United States, healthcare costs are growing at an unsustainable rate, while outcomes continue to worsen, coverage continues to tighten, and efficiency continues to decline. Healthcare spending will represent 1/5th of the US economy by 2032, growing from $4.8 trillion in 2023 to $7.7 trillion by 2032. As the population continues to age and utilization of healthcare services increases, the current mechanisms to cover healthcare costs and enable cost-effective access to care are unsustainable and mandate reform [https://www.healthaffairs.org/doi/10.1377/hlthaff.2024.00469].

More broadly, consumer challenges around finding reliable, actionable guidance on managing their care are worse than ever before. Nearly 8% of all queries to Google are health-related, with general dissatisfaction in how contextualized and helpful responses are to enable consumers to take the right steps to manage their care. Estimates say almost 10% of all queries to ChatGPT are health-related, with consumers hoping for guidance that is directly actionable and relevant to them [https://www.nature.com/articles/s41746-025-01546-w].

The root cause around rising healthcare costs and the key friction points to resolve compose of three key areas:

- There is a disconnect between the front door that consumers use to access healthcare information (i.e. search engines) and tools accessible to them to provide accurate diagnoses and effectively route them to the right care in the context of their clinical need and healthcare coverage (i.e. patient portals). Provider and payor-led tools for consumers to access the healthcare information are at a significant disadvantage in terms of user experience while generalized search engines are at a disadvantage in understanding a consumer's healthcare coverage and medical history to give them tailored, cost-effective, and actionable guidance.
- Payors and providers fundamentally distrust each other – there is a zero-sum notion that underpins the tension between payors and providers which has compounded beyond rate negotiation to extensive claims adjudication cycles, prior authorizations, and sub-optimal network construction practices. Greater transparency systems and new contracting structures are a stepping stone to rebuilding these strained relationships but have been incremental and difficult to scale. Improving the infrastructure and data they rely on to work together is a structural solution to this – better aligning the payor and the provider and eliminating several middlemen that create tension through increased disintermediation. Until this is resolved, there is a structural handicap on effective ways to align incentives at scale.
- 99% of healthcare encounters are suboptimal in matching consumers to the right provider based on complexity, cultural competency, distance, and quality. While there are wrappers designed at fixing this, the root cause lies in how providers are incentivized to steer consumers, existing mechanisms for network construction, and the

tools consumers have accessible to them to intelligently navigate this. The sweeping structural reform need to solve this lies in how payors integrate providers into their networks and how consumers are empowered to navigate and control their own healthcare [Appendix 1].

Novitas is helping to build the next-generation healthcare system where three core principles are true:

1) Incentives are restructured to **enable underwriting and care design at the individual level**, supercharged by artificial intelligence advancements that ensure every person gets the care they need with diagnoses that are more deeply grounded in science than art, enabling our healthcare system to manage risk and improve overall health systematically.
2) Every healthcare encounter is optimized to match the right consumer to the right care at the right time.
3) Payors and providers collaboratively manage a consumer's care and have a high degree of trust in how they collaborate with each other, reducing friction in how care is accessed.

Novitas is a vertically integrated, AI-native healthcare ecosystem that ensures every consumer gets the right care at the right time, transforming the front door of the healthcare system and incentivizing providers to fundamentally modernize how they deliver care.

Vision

The speed of progress in improving foundation models and exponential pace of AI adoption in services-first industries presents a critical reality: **the present-day model of how healthcare interactions take place is going to be entirely overhauled** – from AI driving clinical decision-making and significantly improving diagnosis accuracy to revamping the scale and efficiency of primary care to digitizing most healthcare encounters and leaving a small fraction to still being delivered in discreet, brick-and-mortar clinical encounters.

Broadly, we believe that the future is one where access to high-quality healthcare decision-making is democratized, significantly cheaper, and much higher accuracy as a result of AI advancements – the vision for Novitas is built on this thesis.

The insurance model that covers healthcare costs today, as well as the front door for consumers, is not designed to support this reality. Novitas is the vehicle that will overhaul this constraining infrastructure to unlock how cutting edge AI tools are truly adopted at scale in delivering high-quality, cost-effective, and personalized care. A future where every primary care

doctor can see 1,000 patients a day with AI driving decision-making in a consumer-friendly and highly data-driven way is possible – and Novitas is the infrastructure to unlock this.

In order to drive steerage and fix the underlying economics of healthcare delivery, you ultimately need to be the risk-bearing entity with deep technical integration into the consumer's healthcare navigation workflow, provider's clinical workflow, and employer's benefits management workflow. Beyond being an inefficient use of early-stage capital, there are operational, regulatory, and market competition complexities to building this from scratch without having direct access to meaningful member scale, thus warranting a narrower, software-focused niche that is built around the core premise of Novitas: redefining the consumer healthcare experience at scale. The initial problems to solve are building a large user base by owning comprehensive healthcare navigation for consumers and building consumer trust by driving retention, cost efficiency, and a concierge, personalized experience.

The core foundation of Novitas is a member intelligence and navigation system that has three key components:

1. A consumer-facing app which provides a fully AI-enabled concierge navigation (analogous to an AI-driven healthcare encounter) experience that becomes the front door for consumer navigation of healthcare
2. A benefits and care optimization engine that leverages consumer health queries, healthcare QA data, claims, and medical records to optimize every consumer-facing touchpoint in the context of their history and coverage
3. An analytics system for enterprises (plans, providers, employers) that leverages AI to build optimized benefits packages for users and have significantly improved prediction of healthcare spend

At scale, vertical integration of the entire healthcare stack for a member provides meaningful value, defensibility, and scale. By bringing together the payor, intermediary navigation and consumer front door, and high-performing providers, Novitas has significant leverage in structuring reimbursement models that effectively align incentives and unlock technology enablement throughout the entire care delivery continuum. In the long term, this enables a scalable, technology-enabled, and cost-effective competitor to narrower variants of this model such as Optum or Kaiser Permanente.

Platform

A key thesis for Novitas is that **the entire front door of how consumers interact with the healthcare system will be AI-first and the next 5 years represent a significant window to build a new intermediate.** Novitas' consumer experience is a healthcare navigation app that is accessible free-of-charge through a member's employer or directly on the internet.

The member experience is designed by providing a chat interface built on a healthcare-specific model including: a conversational reasoning system that can answer general healthcare questions, plan-specific and context-based navigation, screenings to become predictive on optimal benefits and healthcare services for each member, embedded agents that complete basic tasks such as requesting refills and scheduling appointments, and clinical workflows that guide members to engage in preventive care.

This member experience will be the equivalent of a personalized health concierge - taking you from diagnosis to completed healthcare encounter seamlessly. Novitas' member experience includes pull and push-based interactions - chat and analytics interfaces where members can ask questions or neatly visualize their healthcare journey combined with periodic screenings that are pushed to members to drive longitudinal engagement.

There are three key differentiators:

1. Embedded agents enable members to go from a Google-like query that is intelligently reasoned through by a healthcare-specific model to completed tasks such as scheduling with in-network providers, finding telehealth services, and closing the loop to ensure members completed recommended actions.
2. Intelligent navigation that stitches together plan / network data with an assessment of clinical assessment, cultural competency, and need to ensure that members are correctly matched to the right provider or clinical action.
3. The underlying data powering the member experience integrates provider directory and pricing data from consumers (and plan / claims data from employers where available) to enable guidance that has context on the specific decision-making that a member is making in the context of their insurance.

A key pillar to success is providing a significant advancement to how consumers navigate basic healthcare questions today - through search engines - as a way to build true retention and trust, ultimately having influence in how members navigate their healthcare decisions and enabling steerage in a way that is personalized, intelligent, longitudinal, and effective. Being exceptionally good at handling ubiquitous consumer tasks (i.e. scheduling a visit, prescription refills, etc) combined with the personalization of the consumer experience enables this in a significant enough way to drive retention.

Enabling us to meet members where they are, enabling Novitas' core member navigation engine to be accessible to members through an app-based experience, SMS, and voice will be critical. The most expensive consumers of employer-sponsored healthcare may have a steep adoption curve for a mobile app - reducing friction in building an initial member base is key.

At scale, Novitas will be the equivalent of a primary care doctor and contextualized healthcare concierge in every single person's pocket. Unlocking this reality with a top-tier consumer experience enables us to become the future front door of the healthcare system in the U.S. and beyond - and more importantly be a meaningful layer of infrastructure in modernizing the healthcare system.

Impact of GTM on Member Experience

The initial GTM approach includes testing a DTC launch and also partnering with self-insured employers. The benefits of a DTC approach are adoption speed and ability to rapidly scale a user base while the constraints are around not having access to plan data by default. The benefits of a B2B approach are having a more defensible consumer experience that is plan-aware but user growth is restricted.

In either case, providing users with incentives that drive them towards the recommended provider selection or healthcare decisions recommended by Novitas will be important to test - this includes ideas such as discounting co-pays or offering access to certain free benefits via partnerships. Figuring out optimal marketing strategies to hook an early user base is an important focus in launching V1.

Direct-To-Consumer App

The design of Novitas' member experience is agnostic to having underlying plan data on members - meaning, we will be launching it as a direct-to-consumer healthcare navigation app as well. In becoming the new, default front door to the healthcare system, we believe that a degree of virality early on is critical - as a result, maximizing access as a healthcare navigation platform that consumers can access with no strings attached is an important strategy to initially building a defensible member base. Additionally, building a member base directly through a DTC launch enables us to 1) get quick feedback cycles on the consumer experience, 2) aggregate large volumes of "front door" data to become smarter at navigation independent of plan data, and 3) get basic user data to better understand how to target the employer-facing go-to market strategy.

User Acquisition

As a result, there are three user activation workflows we initially support:

- Activation via auto-enrollment through your employer
    - In this workflow, the employer directly shares plan and claims data with Novitas to generate and store a comprehensive member profile on the server-side. When the member activates for the first time, this is loaded into their app and is already plan-aware with minimal onboarding friction for the employee.
- Direct-to-consumer enrollment with plan data
    - When a consumer launches the consumer interface via browser or app, they can optionally upload their health insurance policy and complete a medical intake / onboarding workflow. Novitas' engine will aggregate network data and policy information in the background using this and provide a tailored user experience that is plan-aware and contextualized to the consumer.
- Direct-to-consumer enrollment without plan data
    - A consumer can use Novitas via browser or app without any onboarding / credentialing as an intelligent medical concierge that guides them through healthcare questions they may have.

Employer Experience

The employer interface for Novitas is designed to help them personalize benefits packages for members, optimize utilization of healthcare services, and ultimately enable a healthier workforce.

Employers who institutionally partner with Novitas will have a portal from which they can comprehensively track and manage healthcare utilization. As a part of setup, Novitas' infrastructure will integrate with the existing healthcare administration system (either directly or via TPA) to enable the platform to ingest existing plan data and claims data (if available). Using this, the employer interface will have the following functionality:

1. Centralized "air traffic control" to track healthcare utilization and observe trends in employee engagement with health benefits
2. Analytics engine to help predict healthcare spend and provide functionality to evaluate self-insured vs. fully-insured options that are optimal for the organization
3. Recommendation engine to identify benefits that will be high-yield for the employee base with external integrations to various benefits repositories / tools to aggregate an up-to-date and comprehensive list. This enables employers to easily find new benefits that are being launched and understand what's best in the context of their employees.

For self-insured employers, there will additionally be features to:

- Track and predict healthcare spend based off utilization trends
- The ability to unlock employee cash-back / reimbursement / benefits based on optimal navigation and provider selection

As benefits selection modernizes and increases in complexity, along with the trend of employers feeling financial pressure to manage healthcare costs, providing modern infrastructure that employers can use to understand utilization across their employees and have AI-enabled insights into benefit and plan design for their employees has significant market pull right now. Through early conversations with self-insured employers, organizations that have significant first-order P&L impact from suboptimal health insurance and navigation, the market need for this kind of system is clear.

Underlying Infrastructure

One of the most significant gaps in the healthcare ecosystem today is the lack of a "unified" front door that consumers can access - health information is accessed by consumers through multiple interfaces spanning Google / ChatGPT, various provider and payor portals, and the guidance of their provider. The fundamental challenge with this is the lack of shared context required to effectively guide a member through their healthcare decisions in a way that builds trust and context - something that has been historically challenging to design due to technology and integration limitations.

Novitas' key technical differentiator is a set of data that is uniquely stitched together to create a proprietary reasoning base on which members are guided to optimal healthcare decisions. This requires being able to structure various sources of "underlying truth". Novitas' intelligent core blends the following data streams to develop a holistic view of the consumer and their healthcare choices paired with personalized navigation that has high adoption and conversion:

1. Repositories of consumer healthcare questions (i.e. Google, WebMD, Reddit) that can be fine tuned to redirect and contextualize current "front door" interactions
2. Provider directory and pricing data integrated from third-party providers and layered with provider quality data that is scraped from online sources (i.e. Google, Zocdoc, Doximity) to optimally route consumers to the right providers
3. Clinical decisionmaking QA data (i.e. OpenEvidence, UpToDate) to ensure clinical recommendations are personalized and accurate in a way that simplifies how consumers manage their care and stay healthy
4. Epidemiological data (i.e. CDC outbreak maps) to enable topical, real-time preventive care

As Novitas reaches scale, we plan to expand our focus into distribution and data partnerships with health systems, health plans, and employers. Through these channels, V2 of the platform will layer in additional data that contextualizes and validates how healthcare decisions are made:

5. Real-time and historical claims repositories to validate navigation decisions while using FWA and claims adjudication data to optimize encounters
6. Medical records which allow Novitas to build context on a member over time

Being able to stitch together the entire flow of how provider referrals and search engine recommendations translate into concrete healthcare spend to appropriately drive navigation (via ranked referrals), contextualize recommendations to members that are appropriately incentivized (via network analysis and copay incentives), and change the healthcare engagement model from reactive (consumer-driven) to proactive (proactively identifying what steps members should take to manage their health) is the biggest unlock in reshaping how consumers interact with the healthcare system at scale.

Longer term, owning this full continuum is the unlock to building the most intelligent risk-bearing model in healthcare: **being able to underwrite at the individual level and ultimately bend the cost curve of the entire U.S. healthcare system.**

Go-To Market

For Novitas' vision to succeed in a highly competitive landscape, having a product-led DNA is critical. As a result, the early go-to market is designed around two principles: 1) quick feedback cycles and 2) rapidly building meaningful claims flow.

Self-Insured Employers

The initial go-to market hypothesis is small businesses that are currently self-insured or moving into a self-insured model, with an initial target segment of employers with less than 500 employees. There are a few specific reasons for this:

1. Self-insured employers are directly incentivized to optimize navigation and bring down total costs as they have full exposure to unnecessary medical expenses. With this segment, the ROI analysis is clearer and directly impacts budgeting and the organization's P&L with monthly feedback cycles on total claims amount. Fully insured

employers have slower feedback cycles (annual) and are evaluating these decisions based on second-order effects (i.e. employee wellness).

2. Employers with less than 500 employees have a more significant pain point in predicting total claims amount, as most underwriting models are not well designed for smaller populations. Providing a comprehensive suite of tools for managing steerage and predictive analytics based on utilization and navigation trends directly or through TPA partnerships meaningfully enables smaller employers to stay in self-insured models and immediately see reduction in total healthcare expense.
3. Other go-to market segments for Novitas include selling to larger employers, payors, providers, or TPAs and are interesting in the long-term. These channels notoriously have lengthier sales cycles, slower adoption timelines, and overall slower feedback cycles which handicaps our ability to be truly product-led and build an initial user base in a highly capital efficient way. Being able to scale to larger employers will be critical to maximize Novitas' TAM, but is rate-limiting in the early stages.

The bottlenecks to keep in mind with self-insured employers are two-fold: 1) ensuring member adoption (as this tends to be low / difficult for most benefits programs), and 2) sales cycles for self-insured groups. Providing detailed analytics to employers around the immediate month-to-month ROI of improved navigation combined with a true lift with consumers on how simple it is to access care are the core of solving this.

Additionally, partnerships with TPAs, PEOs, and benefits management systems will be critical to both develop pipeline and build scalability into the go-to market over time.

Direct to Consumer

In parallel to partnering with self-insured employers, having a direct to consumer channel for accessing Novitas is key to becoming a true "front door" for consumers and also maintaining a quick feedback cycle to understand what consumer workflows matter most in influencing their healthcare decisions. In launching a DTC experience, building virality while also providing key differences to an existing chat-based experience will be important. One mechanism we are evaluating to do this is embedding content that is co-branded with a major academic medical center (i.e. Mayo, Stanford) to build trust and provide consumers access to the best quality medical care in their pocket. By having task-specific agents, with an initial focus on eligibility checks, scheduling, and virtual consults, embedded in Novitas' consumer experience, they can use it as a healthcare concierge that is an independent, trustworthy medical advisor that ensures they're always taken care of.

Depending on the success of an initial DTC launch and capital efficiency / defensibility of employer-based distribution, we expect to explore a DTC-first approach to scaling Novitas in the long-term with employer and health system partnerships as secondary growth avenues.

Market Positioning

Once you have a significant user base and meaningful influence over consumer decisions, you have the critical positioning necessary to dramatically change the way consumers make decisions about their care and unlock a future healthcare system that brings down the unit economics of delivering care.

Market Sizing - B2B

Phase 1 – Self-Insured Employers (≤500 employees): While small firms are less likely to self-fund than large ones – roughly 16% of small firms and 32% of mid-sized firms self-insure at least one health plan – they represent a small niche that acutely feels the problem Novitas solves. Based on U.S. Census data, there are ~5.7–5.8 million employers in the U.S., the vast majority of which are small businesses. Approximately 53% (~3 million) offer health benefits to employees. As a result, the subset to focus on in Phase 1 includes approximately ~300,000 small employers and ~25,000 mid-sized employers.

In a total market of approximately 325,000 potential customers (self-insured employers ≤500 lives) which employ approximately 10-12 million workers, even Novitas' Phase 1 focus can reach significant scale.

Phase 2 – All Employers Offering Health Insurance: Approximately 95% of all employers in the U.S. over 50 employees offer insurance - equating to nearly 3 million employers and approximately 91% of all U.S. employees (154 million employees are provided health insurance through their employer). Unlocking this market segment as Novitas scales will be critical to building an enduring business that can transform the landscape of health insurance in the United States.

The pricing model for employers is a small base activation fee with a variable PEPM scale-based component. We need to be very particular about initially staying narrow in our focus and optimizing around owning navigation for members - the north star in doing so is users who are activated into the app and the pricing structure / revenue metrics should be representative of this. While more pricing details will have to be tested as we go to market, a basic example is as follows:

- *Small employer (100 employees):* Annual base fee $30,000 + $10 PEPM. With 100 employees, the PEPM portion is $1,000/month. Total annual cost ≈ $42,000. The pricing model will depend on market feedback but needs to be at a meaningful discount to manual / TPA-based navigation with a strong >5x ROI story.

These assumptions give us a concrete view of Novitas' projected TAM:

Segment	Total Organizations	Total Lives	TAM
Small Self-Insured (Phase I)	~325,000	~11 million	$2.13B
Employer-Sponsored Market (Phase II)	~3 million	~ 154 million	$25.98B

A purely direct to consumer approach would represent a different TAM which is harder to estimate as there are various monetization channels to explore (i.e. advertising, co-branded partnerships, payor partnerships). If we are successful in scaling a DTC motion and are clearly taking market share in owning consumer navigation of healthcare, we are confident there is significant long-term value.

Competitive Landscape

There is a long list of companies that have tried to solve navigation while become a new "front door" for the healthcare system without much success - and there is a crowded landscape of present-day companies that Novitas has overlap with. Specifically, we cut between two axes of companies - pure navigation systems and full-stack, software-enabled care delivery providers. While we don't directly compete with either, based on the scope of our platform, and sit between these two sets of companies, there is some overlap and lessons that we are applying to building Novitas with a brand new model for consumers to interface with the healthcare system.

<u>Pure Navigation Systems:</u>

This includes companies such as Accolade, Apree Health (Castlight), Solace, and HealthJoy. Every single company in this space has a manual, advocate-driven component that focuses on the healthcare interaction once a consumer has decided to interact with the healthcare system and pursue care. These approaches have struggled to scale largely because they are only able to capture one segment of the entire healthcare experience a consumer has to navigate - they don't represent a front door that a consumer starts with and don't have integrations into all the various ecosystems necessary to seamlessly complete tasks for members. As a result, companies in this space have struggled immensely with go-to-market because they require having some degree of integration with payors, providers, *and* employers while also having services-heavy operations that are difficult to scale in a way that is 10x better for consumers

than current tools already accessible to them. We have taken these lessons and implemented a few key differences in our model:

1. By focusing on being the true front door to where consumers begin to navigate the healthcare system (search engines) enables a much larger top-of-funnel for user acquisition while also allowing us to have control of the triage process so that we can influence the healthcare utilization decision made by the consumer and nudge them in the right direction more effectively.
2. Having sophisticated embedded agents built using modern AI tooling enables us to actually complete tasks entirely on behalf of consumers (and have higher conversions on effective navigation / care recommendations) and follow-up with members to ensure they're managing their care correctly.
3. Integrating the range of data sources outlined above gives us a meaningful product differentiation in combining breadth and depth and working with a very broad user base - we can flex from engaging in preventive care based on outbreaks to algorithmically (not manually) guiding members to a provider that is optimized for their need based on cost, quality, distance, and competency.

The ability to have a comprehensive health concierge that can effectively go from answering a basic healthcare question to do intelligent care routing to proactively help a consumer manage their health is a very different and more end-to-end, software-driven approach to other navigation systems in the market today.

Full-Stack Care Delivery:

Novitas is not directly competitive with care delivery providers such as Included Health, HealthTap, K Health, and several other telehealth companies that offer AI-based navigation paired with a clinical visit. Notably, Novitas is not and does not want to be a care provider in the near-term - to build trust with consumers in helping them interact with the healthcare system, we feel strongly that you must be a neutral third-party that guides the consumer to the right healthcare decision. Consumers inherently don't trust payors or providers as a result of alternate incentives or business models they have in holistically giving the right guidance.

Further, in crafting our business model, being able to be a software-only provider in the near-term is valuable to have the most efficiency and "lean" structure in scaling to a large user base that covers significant breadth.

Notably, companies that are adjacent to Novitas include Doctronic, Your.MD, Ada Health, Ping An Good Doctor, and others. Beyond technical differences outlined above, Novitas is taking an approach that is truly end-to-end in interfacing between the consumer and payors/providers while also strategically partnering with health systems, plans, and employers to build

distribution and data defensibility in the long term. In a market that is still largely untapped today and actively growing (in the U.S. and internationally), there is significant whitespace to fill if we execute quickly and effectively.

Scale

In the long term, consumers should be able to affordably access routine healthcare visits without the constraints of insurance, with insurance reserved for serious medical events that have significant expenditure (in essence eliminating the need for indemnity plans and transitioning to a modified form of stop loss insurance that just focuses on "catastrophic" coverage). Having an AI-first, consumer-friendly system that functions as the concierge and primary care experience while routing to in-person visits for more meaningful or advanced types of care enables this while also providing the chassis for redesigning the entire premise of health insurance in the United States. Novitas is positioned to capture the rapidly reducing cost of delivering care that is facilitated by foundation models and provide the entirely new operating model for the healthcare system on top of it.

Specifically, optimized navigation and a sticky consumer front door represent two highly defensible moats that provide a launching pad to several major business models at scale:

1. Becoming a consumer-first, openly accessible healthcare system
    a. With a widely adopted front door that is helping consumers concretely take actions to manage their care, Novitas has potential to build an entirely new AI-first healthcare system that consumers can access anywhere and have a complete healthcare encounter on. This includes providing optimized access to care through health plan partnerships as well as bespoke "subscription" products similar to a direct primary care model that allow consumers from anywhere in the world to get access to leading medical professionals for healthcare concerns that are intelligently triaged and routed with a high degree of precision.
    b. In becoming a completely new healthcare system that consumers interface with, Novitas is the infrastructure on which healthcare shifting into the home and more scalable, virtual-first panel sizes happens at scale (compared to many attempts at doing this incrementally or in fragmented ways).

2. Vertically-integrated payor
    a. Ultimately having direct influence over navigation provides the most important component at predicting healthcare spend - and providing modern insurance products that match an AI-first model for delivering care. An insurance carrier built on Novitas would be fully technology-enabled, with AI-based tools used across all key plan functions: underwriting, claims adjudication, policy

management, utilization management, compliance and regulatory oversight, analytics, enrollment and eligibility management, and payments. This tooling can enable a 10%+ improvement on accuracy and 50%+ improvement on efficiency. There's meaningful operational efficiency to gain from designing a technology-first plan using tools that exist today, rethinking the basis of how insurance companies operate.

3. Building depth in employer insights around benefits selection and healthcare utilization to become a modern benefits and insurance administrator for employers
    a. While TPAs are likely to get as artificial intelligence replaces basic insurance administration operations that currently exist, there is a clear market need for better intermediaries that manage the full breadth of benefits for employers while enabling self-insured options, tracking and predicting benefits spend, and becoming the central platform for managing all employee benefits.

Building V1

The full breadth of Novitas is very large, by design, and getting the core foundation right will be critical to building a lasting business.

Company Infrastructure

Initially, it's critical for Novitas to have a truly product-led DNA with a high degree of technical excellence in building a navigation experience that is more comprehensive, personalized, and easy-to-use than existing front doors to the healthcare system. Maintaining quick feedback cycles in improving the consumer application and scaling it will be critical to the most important principle for V1: build a rapidly growing user base that relies on Novitas' guidance to manage an end-to-end healthcare experience. With clarity on what the core infrastructure and required data to build an industry-defining experience, rapidly experimenting with DTC and B2B angles to also get clarity on the optimal GTM for V1 will be an important early execution test.

As a result, the founding team needs to have three core pillars:

1. Technical excellence that has worked at the forefront of AI research with significant exposure to foundation models, is comfortable with large, convoluted, and hard-to-aggregate datasets, and can iterate rapidly
2. Operational excellence that has extensively worked with healthcare enterprises and can build an exceptional organization while maintaining a fast pace of GTM iteration

3. Consumer design excellence that can bring the best consumer experiences across the technology industry into healthcare

As a software-only initial focus, we can stay very lean and capital efficient (relative to building an enterprise-only business or full-stack insurance platform) in getting to an exceptional consumer experience while also rapidly iterating through various GTM channels to understand what provides the best leverage in building a core user base.

Why Now

There are three major trends right now that have created a pressing market need for Novitas:

1) Consumerization of healthcare: As technology continues to change into how healthcare is delivered, consumers have greater optionality than ever in how they access healthcare - ranging from how they find doctors to new payment models to the breadth of services accessible to them. While this has broadened access, this has created further fragmentation in how people interact with the healthcare system and amplified the need for navigation that empowers consumers to manage their health from a single interface and make informed decisions.

2) AI enablement of healthcare: Reasoning models have improved rapidly and made it so that the majority of clinical decisionmaking can be done effectively by AI-based systems with supervision by clinicians (~80% can be done by AI today compared to ~20% only 5 years ago). As these models continue to improve, they will democratize access to clinical reasoning and the cost of accessing high-quality healthcare will significantly decrease - giving consumers the right channels to access this through will be key to becoming a major part of the healthcare ecosystem of the future.

3) ICHRA and wallet share of commercial insurance: The tension between employers and employees about the quality of healthcare coverage accessible to them combined with the proliferation of health-related benefits has rapidly pushed employers to look for alternative ways to manage healthcare spend and give consumers better infrastructure to manage their health in a way that is personalized and high-quality - most apparently evidenced by the adoption of ICHRA and the increasing percentage of employers of all sizes (but notably small and mid-size) exploring self-insured options. Giving them better navigation tools to predict healthcare spend while also balancing costs of delivering excellent employee experience and benefits has tremendous market need today. As this trend continues, the employers that are able to offer high-quality navigation and predictability around healthcare utilization will be best equipped to retain their employees, manage overall costs, and maintain a healthy workforce.

Appendix 1 - Steerage Data

Key Statistics on Inefficient Patient Steerage

- Misguided Post-Hospital Care Choices: Annually, over $250 billion of Medicare spending is directed to post-hospital care providers selected more for convenience than quality. Notably, 46.8% of patients are discharged to providers that are outperformed by at least five others within a 15-mile radius. Only about 15% of patients end up at top-quality providers nearby. This indicates that outdated discharge processes often prioritize proximity over performance, leading to increased costs and poorer outcomes. [https://time.com/5027083/uncle-alzheimer-battle-inspired-me-to-disrupt-healthcare/]

- Overuse, Underuse, and Misuse of Care: Approximately 30% of U.S. healthcare spending is attributed to poor-quality care, encompassing overuse, underuse, and misuse. This inefficiency results in hundreds of billions of dollars in unnecessary costs annually. [https://pmc.ncbi.nlm.nih.gov/articles/PMC4115298/]

- Impact of Vertical Integration: Patients whose primary care physicians are affiliated with large health systems experience a 22.6% increase in specialist visits per year. This suggests that vertical integration can lead to higher utilization and spending, potentially without corresponding improvements in care quality. [https://www.fiercehealthcare.com/providers/newly-integrated-pcps-steer-patients-toward-systems-higher-utilization-spending-study/]

- Unnecessary Medical Services: Physicians estimate that about 20% of medical care—including tests, procedures, and medications—is unnecessary. This overutilization not only inflates costs but can also harm patients by exposing them to unwarranted interventions. [https://www.cedargate.com/resources/our-fee-for-service-healthcare-system-is-failing-patients/]